

The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, disease burden, and the gap between need and care.

Global Data | 2023

Source: Our World in Data — Mental Health (Dattani et al., 2023) | Data: IHME, Global Burden of Disease (2025)

For: Public Health Policy Analysts & Mental Health Program Planners

Hundreds of Millions Affected

Major depression affects a substantial share of the global population across all demographics.

1 in 3

Women

will experience major depression in their lifetime

1 in 5

Men

will experience major depression in their lifetime

SPECTRUM OF IMPACT

- **Depression & Anxiety**

High prevalence

- **Schizophrenia**

Severe disability

- **Bipolar Disorder**

Chronic episodes

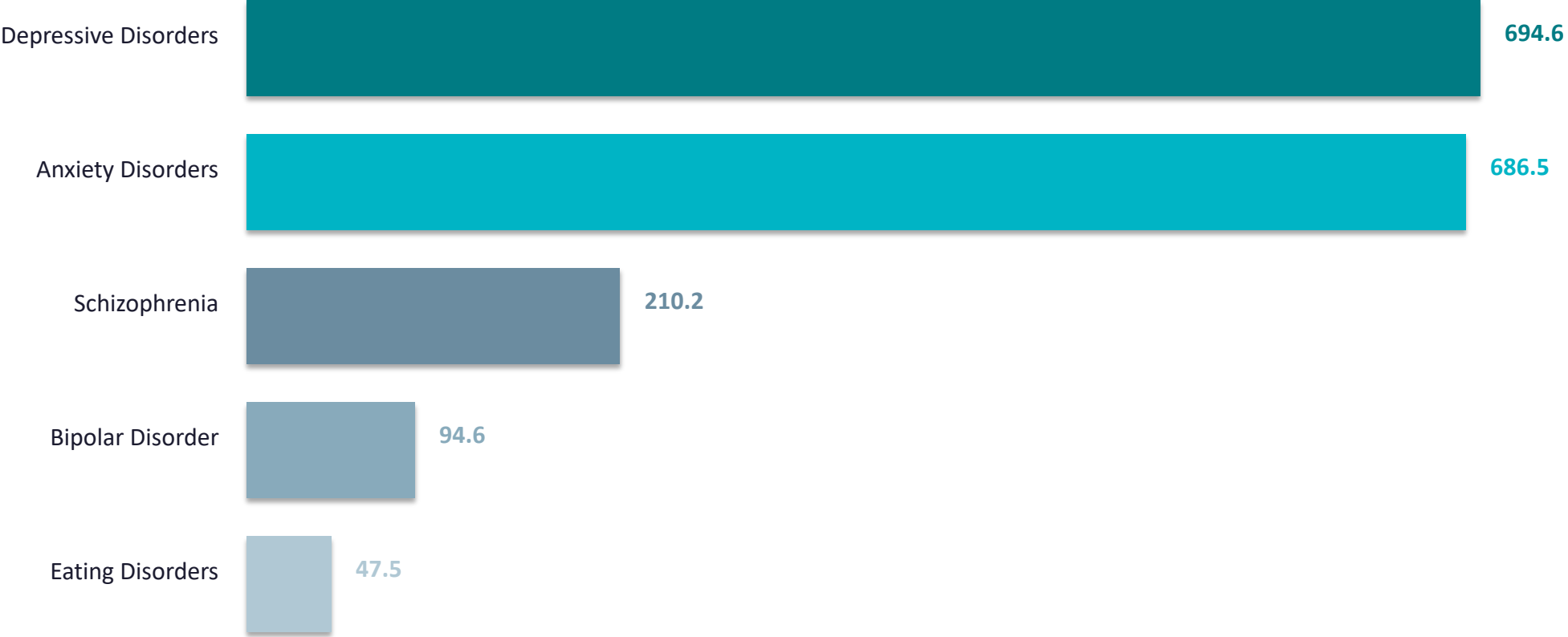
- **Eating Disorders**

High mortality risk

Mental illnesses are treatable and their impact can be reduced — yet treatment is often lacking or of poor quality. Understanding scale is the first step toward effective policy.

Depression and Anxiety Dominate the Global Mental Health Burden

DALYs per 100,000 people — World, 2023 | Source: IHME, Global Burden of Disease (2025)

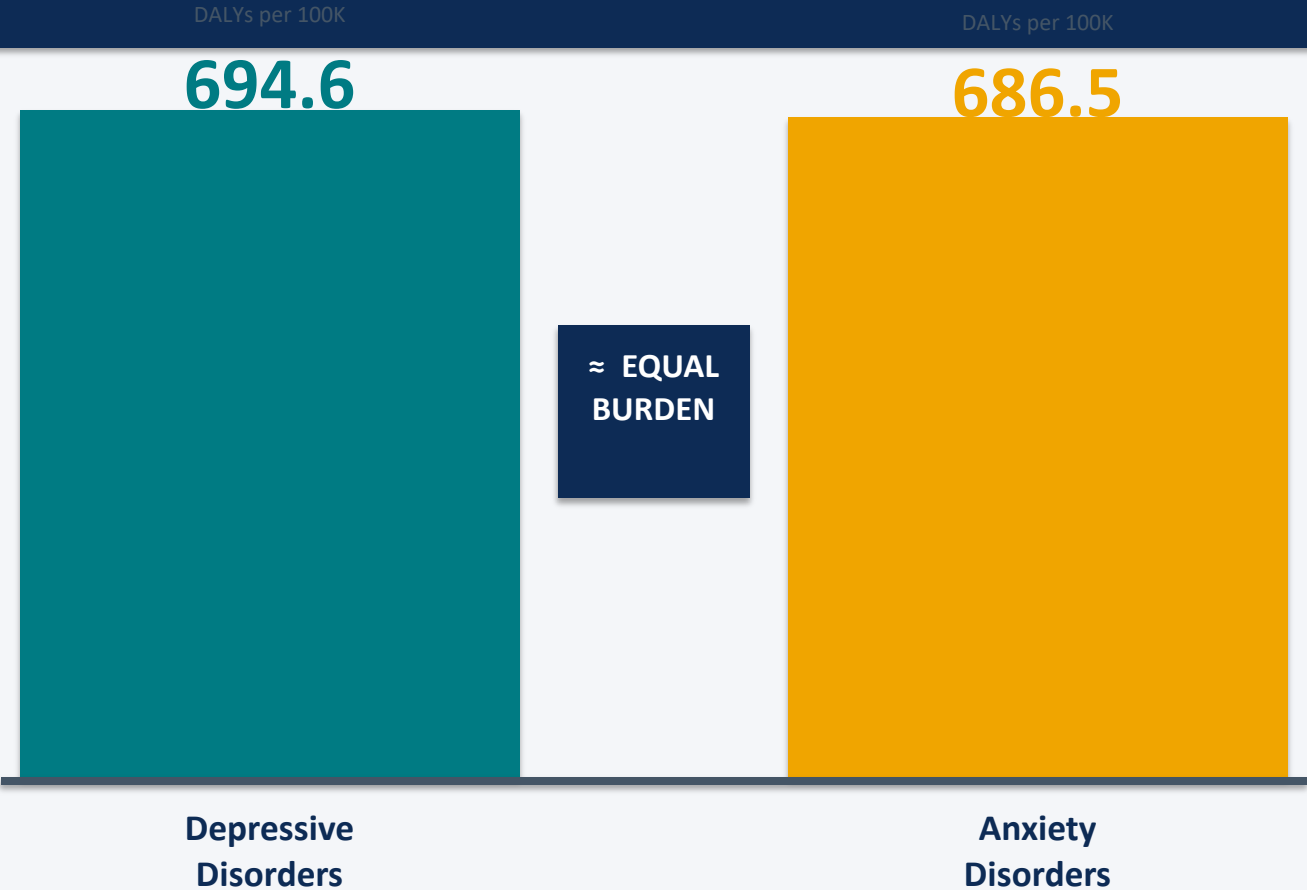


Depression + Anxiety account for ~80% of total burden across these five categories.

DALYs per 100,000 population

Depression and Anxiety Carry Nearly Equal Disease Burden

Side-by-side comparison of DALYs per 100,000 — World, 2023

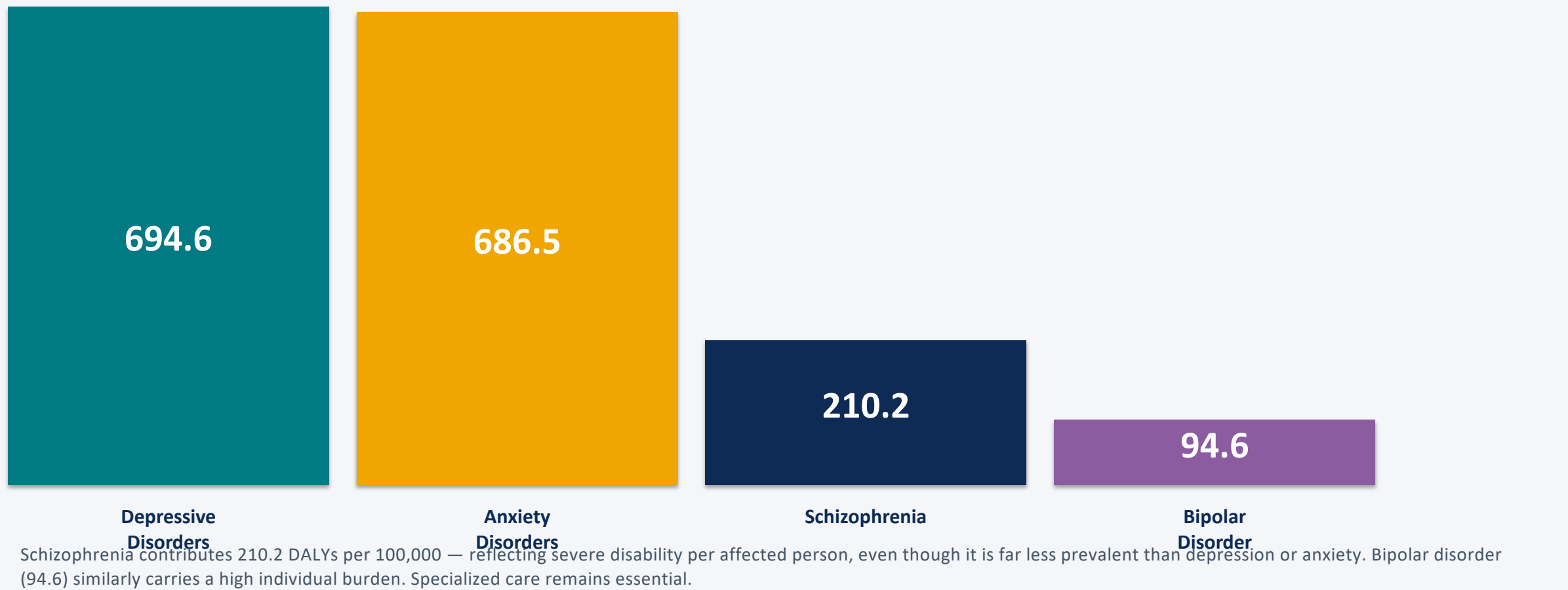


What does this mean for policy?

- Together, depression and anxiety represent ~80% of the mental health disease burden across the five major categories.
- Near-equal burden means interventions targeting anxiety are just as critical as those for depression.
- Combined, they affect far more people than schizophrenia, bipolar disorder, and eating disorders combined.
- Scaling evidence-based treatments for both conditions should be a top priority.

Schizophrenia and Bipolar Disorder: Lower Prevalence, Severe Individual Impact

DALYs per 100,000 — World, 2023 | Disproportionate disability per affected person



Treatment Is Available — But Often Lacking or Poor Quality

The treatment gap: effective interventions exist, yet billions go without adequate care



STIGMA

Many people are uncomfortable disclosing mental health symptoms — to healthcare providers or even family and friends.



ACCESS

Treatment is often lacking, especially in low- and middle-income countries where resources are most constrained.



QUALITY

Even where treatment exists, it may be of poor quality — failing to meet evidence-based standards of care.



UNDERREPORTING

Discomfort with disclosure makes true prevalence difficult to estimate. The actual burden is likely higher than reported.

Good data is essential — understanding how, when, and why mental illnesses occur, and how many people are affected, is critical to designing effective, safe treatment programs.

How People Cope: From Medication to Faith to Family

Global survey data reveals wide variation in coping strategies for anxiety and depression



Prescribed Medication

Common in higher-income settings where healthcare access and pharmacological treatment are widely available.



Religious or Spiritual Activities

Prevalent in many regions worldwide. Spiritual practices are a primary coping mechanism for millions.



Talking to Friends or Family

A widely reported coping approach across all income levels and cultures — social support as medicine.

Policy implication: The diversity of coping strategies reflects cultural, economic, and healthcare system factors. Effective interventions must be culturally sensitive — a one-size-fits-all approach will not address the global treatment gap.

Key Takeaways: Priorities for Mental Health Policy

01

Depression & Anxiety Are the Dominant Burden

Together they represent ~80% of mental health DALYs (~695 and ~687 per 100K). Scaling evidence-based treatment for these common disorders must be the top policy priority.

02

Schizophrenia & Bipolar Carry Disproportionate Individual Burden

At 210.2 and 94.6 DALYs per 100K respectively, these conditions cause severe disability. Specialized care must be maintained even as resources shift toward high-prevalence disorders.

03

Treatment Exists — But Access and Quality Remain Inadequate

Stigma, geographic barriers, and quality gaps leave millions without care. Health systems must prioritize expanding reach and improving quality of existing treatments.

04

Stigma Suppresses Help-Seeking and Distorts Data

Underreporting means the true burden is likely higher than reported figures. Anti-stigma campaigns and anonymous reporting mechanisms are essential components of any strategy.

05

Culturally Sensitive, Locally Adapted Interventions Are Essential

Coping strategies vary widely — medication, spiritual practices, social support. Policy must reflect this diversity rather than imposing a uniform approach.